Consolidation Report

Healthcare Financial / RCM Platform & Hospital IAM Gateway

1. Documents Reviewed

- Market Research
- Healthcare Finance DDD
- Healthcare Finance HLD
- Healthcare Finance LLD
- Healthcare Finance ERD
- Hospital IAM BRD
- Hospital IAM PRD v3.4
- Cross-Document Analysis Report

2. Overall Assessment

The documents are generally well written and each document set follows a logical structure. The IAM BRD and PRD are strongly aligned. The Finance documents also have a consistent overall business and architecture story. However, the two systems are currently documented as separate systems and some Finance details are not synchronized across the DDD, LLD and ERD.

Overall Status: GOOD FOUNDATION – CONSOLIDATION REQUIRED BEFORE FINAL IMPLEMENTATION

3. What Is Correct and Connected

Document Flow	Status	Simple Finding
IAM BRD → IAM PRD	Good	Business requirements and technical requirements are closely aligned.
Market Research → Finance DDD	Good	The RCM problem and proposed solution are consistent.
DDD → HLD	Good	The main bounded contexts and architecture are carried forward.
HLD → LLD	Good	The modular-monolith, async processing and integration approach is continued.
DDD/HLD → ERD	Mostly Good	The main financial entities and journey are represented.
IAM → Finance	Needs work	The Finance HLD does not clearly connect to the already-defined IAM Gateway.

4. Main Loopholes / Items to Fix

1. Status mismatch: Invoice, Claim and Pre-Authorization use different status names in DDD, LLD and ERD. Create one canonical status list and use it everywhere.
2. Aggregate mismatch: The DDD's detailed aggregate list does not fully match its summary. Payment and Adjudication need to be explicitly confirmed as aggregates or entities.
3. IAM–Finance connection: If both systems belong to the same hospital ecosystem, the Finance HLD should explicitly use the IAM Gateway instead of leaving authentication as TBD.

4. Finance MVP is large: The Finance MVP combines most of the Market Research roadmap phases. Add clear phasing, priorities, team assumptions and deferred features.
5. Adjudication wording: Clarify that the platform records/interprets payer decisions and does not independently make final adjudication decisions.
6. IAM BRD detail: The BRD should include the PRD's 12-hour co-sign deadline and automatic suspension rule for provisional clinical profiles.
7. Compliance wording: Compliance and retention statements should be reviewed and approved for the actual deployment jurisdiction.
8. Identity mapping: Define how IAM identity, patient identity and Finance patient records are linked.

5. Recommended Final Flow

The final documentation should clearly follow this structure:

Market Research → Business/Product Scope → DDD → HLD → LLD → ERD

IAM should be treated as a shared platform capability and its integration with the Finance platform should be explicitly documented.

6. Final Conclusion

The documentation is not fundamentally wrong. The main architecture and business direction are sound. The remaining issues are mostly consistency and integration issues rather than a need to redesign the system. Before development starts, reconcile the Finance status values and aggregate definitions, define the IAM–Finance integration, and clarify Finance MVP phasing and scope.

After these changes, the documents will provide a much clearer and safer implementation baseline.